As a union of 1.8 million members working in education, healthcare and public services, the AFT is deeply committed to investing in innovations to improve care while also protecting patient safety and the clinical judgment of health professionals. Our union has not taken a stance broadly against artificial intelligence; rather, we have embraced the opportunity of new technologies and advocated for meaningful guardrails that ensure AI and other data-driven technologies are safe, effective, therapeutic tools in all clinical environments.

We offer these comments in response to the Department of Health and Human Services Request for Information to “Harness Artificial Intelligence to Deflate Health Care Costs” to highlight the perspective of the more than 250,000 health professionals we represent and the patients and communities they serve. Specifically, we are responding to questions six and nine.

In response to Question 6: *Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve healthcare outcomes, give new insights on quality, and help reduce costs?*

AI use in healthcare is not just a technical innovation, it is also a political, ethical and labor issue. Potential benefits to patients, communities and healthcare workers depend on transparency, regulation, and workers’ rights to ensure that AI enhances , *not replaces*, the human foundations of care.

Where AI tools have met or exceeded expectations, success has come from narrow, task-specific applications. These tools perform best when they are well-trained, transparently evaluated, and used under the supervision of qualified healthcare workers who have the autonomy to exercise clinical judgment. In these settings, AI may be able to reduce administrative burden and support clinical decision-making without compromising the quality of care, but only if health professionals are involved in every step of the process from decision to implementation, and evaluation.

AI tools and other data-driven technologies have the potential to pose life-threatening risk to patients when deployed in this high-risk setting without evidence that demonstrates both safety and effectiveness prior to deployment. Further, without meaningful engagement from healthcare workers or when the purpose of these tools is to facilitate deskilling, speed-up activities, automate management, and surveil workers, AI has an increased likelihood of adverse impacts on patient care. It is essential that workers have the protected right to bargain over the decision to implement any technology that impacts patient care, working conditions and the delivery of patient care. Healthcare technologies that are deployed primarily to facilitate cost-cutting measures may suffer from low adoption, poor integration into clinical workflows, increased administrative burden, and risks to patient safety.

The greatest potential for improved outcomes, new insights on quality, and cost reduction lies in novel, worker-centered applications that can improve diagnostics and enhance treatment outcomes. Benefits such as medical imaging, drug discovery, personalized medicine and administrative tasks depend on strong guardrails, transparency and enforceable worker voice. In high-stakes settings such as healthcare, safety and effectiveness must be prioritized over any potential cost savings to the business. AI tools are most likely to improve care when healthcare workers have meaningful input into how technologies are designed, selected, implemented and evaluated. When workers are involved early and continuously, AI is more likely to deliver sustainable improvements in patient outcomes and healthcare workers' experiences.

In response to Question 9: *What challenges within healthcare do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?*

Patients and caregivers have significant concerns about how AI is being introduced into clinical care, particularly when human judgment, privacy and accountability are weakened or threatened. AI tools overseen by qualified healthcare workers have the potential to benefit healthcare delivery for patients and healthcare workers alike, but only when they are rooted in human foundations of care and used with human oversight.

As AI use in healthcare grows, existing challenges faced by patients, caregivers and healthcare workers may intensify, and new challenges are likely to emerge. Addressing these concerns requires meaningful worker involvement at every stage of AI development, procurement, deployment and use; clear assurance that human professional judgment remains the ultimate decision-maker; strong employer accountability; and safeguards to ensure technology makes care more humane rather than more burdensome. Without these guardrails, AI risks exacerbating problems related to data privacy, transparency, bias, trust, labor rights and environmental harm.

Lack of transparency and accountability: Many AI models operate as "black boxes," making it difficult for healthcare workers to explain how clinical conclusions are reached. When providers cannot explain why an AI tool flagged a cancer diagnosis or treatment recommendation, informed consent is undermined and professional judgment is weakened. Increased data sharing across systems also raises the risk of breaches and misuse; even so-called de-identified data can be re-identified when combined with other datasets, exposing patients to discrimination based on sensitive conditions such as HIV status or mental health history. The use of "ambient listening" technologies to record patient-provider conversations further heightens privacy risks and may cause patients to withhold crucial information unless patients and workers can ensure data privacy and that employers are clearly liable for data protection and misuse.

Additionally, the use of AI models to review care plans, insurance claims or prior authorization requests risks denying essential medical care to patients without a clear explanation. Even if decisions about claims rendered by an AI model are reviewed by qualified clinicians, without insight into how the model operates, the clinician cannot meaningfully review the model's decision.

Bias and health inequities: AI systems reflect the data used to train them, and existing datasets frequently underrepresent marginalized communities. Without continuous oversight by healthcare professionals, AI tools may generate confident but inaccurate outputs, leading to misdiagnosis or delayed treatment for patients who fall outside dominant training populations. Human clinical judgment is essential to identify and correct these failures before they cause harm.

Preserving human decision-making and improving working conditions: Patients may fear over-reliance on algorithms that override clinician expertise, or under-reliance that shifts the burden of validating AI outputs onto already overextended healthcare workers. And in financially strained systems, AI is often introduced as a substitute for human labor rather than as a support, contributing to understaffing, increased workload, and a further erosion of the human foundations of care.

Environmental impacts: Large-scale AI infrastructure contributes to pollution and resource strain, with some facilities emitting thousands of tons of nitrogen oxides annually, often in marginalized communities already facing disproportionate health burdens. Employers must be held accountable not only for clinical outcomes, but also for protecting workers' rights, patient privacy, and community health as AI technologies are adopted.

Liability: Clear lines of liability and safeguards are essential. Employers and vendors must remain fully accountable for harms caused by AI systems used in clinical care. Protections for healthcare workers to question, override or disregard AI-generated recommendations that conflict with their professional judgment should be explicit, enforceable and embedded in written protections such as collective bargaining agreements, workplace policies, and clinical governance structures, so that exercising professional judgment never exposes workers to retaliation or liability. Federal policy should reinforce and uphold these protections as a condition of AI deployment, ensuring that new technologies strengthen safe, ethical and human-centered care rather than undermine it.